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Before Family Risk Becomes a Health Crisis

Trustworthy Premarital Health Education as Primary Prevention Infrastructure

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Yaoharee Lahtee^{1,2}

¹ARAYA Nikah Social Enterprise Co., Ltd., Bangkok, Thailand • ²Independent Researcher, Thailand

ORCID 0009-0005-3861-0626 • arayawedding@gmail.com

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Abstract (accepted, preserved verbatim)

Background. A family should be a space of physical, psychological, and economic safety. Whether it becomes one is shaped at the transition into marriage, a governance threshold where consent, authority, help-seeking, and access to protection are organized before a household is formed. When that threshold fails, the health consequences include violence-related injury, psychological distress, reproductive harm, and avoidable crisis-service use. Prevention frameworks such as WHO's RESPECT cover relationship skills, empowerment, safe environments, and services, yet this transition remains an underdeveloped intervention point. **Objective.** To propose Trustworthy Premarital Health Education (T-PHE) as primary prevention infrastructure that increases each participant's control over marriage-related health decisions within safeguarding and referral boundaries. **Methods.** This practitioner-led, document-informed conceptual synthesis drew on the governance and safeguarding procedures of ARAYA Nikah, a Muslim-led social enterprise registered in Thailand, and on its premarital education programme for Muslim and intercultural couples (over 600 participants since 2024). Recurring concerns in these sources were synthesized, through a culturally rooted ethic of limited knowledge, entrusted responsibility, human dignity, just boundaries, and repair, into a programme theory with six safeguards, a failure criterion, and outcome domains for prospective testing. Sources came from one organization; no effectiveness testing was undertaken. **Results.** T-PHE is trustworthy only when it protects informed choice rather than relationship continuation. It rests on one mechanism: increasing each participant's control over whether to proceed with, delay, or refuse marriage and over when and how to seek help. Six safeguards protect that choice: epistemic humility, cultural intelligibility, affected-person visibility, dignity and consent, bounded authority and referral, and repairability. Educators must make power asymmetries and coercion visible, keep culture and religion from becoming unchecked authority, and refer beyond their own competence. Communication and conflict training is not universally benign. Where violence, fear, or coercive control is suspected or disclosed, joint work must stop pending confidential assessment and must not resume where coercive control is confirmed; survivor-centred support and qualified referral take precedence. T-PHE fails if it raises knowledge scores, attendance, satisfaction, or marriage completion while leaving informed choice, recognition of coercion, reproductive and mental-health literacy, safe help-seeking, and accountable referral unchanged. **Conclusion.** Premarital health education should be recognized, governed, and tested as primary prevention infrastructure, not optional relationship advice. Public health should enter before the family becomes a site of clinical rescue.

Keywords: premarital health education; health promotion; intimate-partner harm; primary prevention; safeguarding

Claim boundary. This paper reports a conceptual synthesis and a programme theory. It is not an effectiveness trial, a systematic review, a qualitative study, a Delphi exercise, an implementation study, or a validated intervention. The accepted abstract is the claim ceiling: nothing in the full text claims that T-PHE improves, prevents, or reduces any outcome. Nothing here is clinical advice. Suspected or disclosed violence or coercive control requires qualified, survivor-centred services.

1 Introduction

1.1 The family as a health environment

The family is not only a private relationship or a cultural institution. It is a durable environment in which health risks and protections are distributed: emotional safety, material resources, reproductive autonomy, access to care, legal documentation, social support, and the ability to seek help. Intimate-partner violence (IPV) is therefore a public-health concern rather than a private moral failure. WHO identifies serious physical, mental, sexual, and reproductive consequences, includ-

ing injury, depression, anxiety, post-traumatic stress, unintended pregnancy, sexually transmitted infections, miscarriage, stillbirth, preterm delivery, and low birth weight, and assigns health systems a role in protocols, training, survivor-centred services, multisectoral referral, and prevention [238].

Global Burden of Disease estimates for 2023 attribute 18.5 million disability-adjusted life-years (95% UI 8.74–30.0 million) to IPV among females, with a substantial mental-health component [1]. In Thailand, the Department of Women's Affairs and Family Development reported an average of 42 persons affected by violence

per day in 2024 across recorded cases involving children, women, or families [239]. This is a recorded-case statistic, not a prevalence estimate, and it is not Muslim-specific. These data establish the seriousness of the problem and the need to examine upstream intervention points; they do not establish that a premarital programme prevents IPV.

1.2 Marriage as a governance threshold

The transition into marriage is usually treated as an educational moment. This paper reframes it as a *governance threshold*: before a household is formed, arrangements are made about authority, consent, documentation, residence, finance, reproduction, conflict, help-seeking, and the legitimacy of outside intervention. A programme that occupies this transition already structures a decision environment—the sequence of activities, the presence or absence of private conversation, the wording of religious or cultural guidance, documentation practices, referral options, and the consequences of pausing. The ethical question is therefore not only *what should couples know?* but *whose interests does the programme protect when participants disagree; can each person speak privately and safely; can a participant delay or refuse without being treated as a programme failure; what happens when fear or coercion appears; where does education end and clinical, legal, religious, or crisis response begin; and how can a mistaken decision be corrected?*

1.3 Why existing content is not enough

IPV is not a marginal risk in the population premarital education addresses; it is among the largest measured contributors to disease burden in women of reproductive age. The 2023 Global Burden of Disease modelling study estimated that 608 million (95% UI 518–724 million) women aged 15 and older had ever experienced IPV, contributing 18.5 million (8.74–30.0) DALYs, and ranked IPV as the fourth leading risk factor for DALYs among women aged 15–49, with anxiety disorders and major depressive disorder the leading attributed outcomes [1]. A separate WHO/LSHTM review, pooling 366 studies across 161 countries, estimated that 27% (95% UI 23–31%) of ever-partnered women aged 15–49 have experienced physical or sexual IPV in their lifetime, with the exposure already present in 24% (21–28%) of women aged 15–19 and 26% (23–30%) of women aged 19–24 [2] — before or around this population’s typical marriage age. Regional data confirm marriage itself is not protective: Indonesian surveys found 12.53–17% of women married before age 18 [3, 4], and among Rohingya women in Cox’s Bazar, child marriage was associated with more than twice the odds of justifying husband-perpetrated abuse (AOR 2.71, 95% CI 1.78–4.11) and with substantially higher odds of past-year IPV (AOR 1.72, 95% CI 1.13–2.61) [5].

Despite this burden, the existing evidence base for premarital and marriage-relationship education (MRE) is narrow: built almost entirely to detect effects on relationship satisfaction, communication skill and, at most, marital stability — not decision control, coercion recognition, or safe help-seeking. The largest MRE meta-analyses

report effect sizes for relationship quality ($d \approx 0.30$ – 0.36) and communication skill ($d \approx 0.43$ – 0.45) [6]; the most current synthesis of a government-funded programme found small effects on relationship quality ($d = 0.114$) and skills ($d = 0.132$) and non-significant effects on stability, parenting and child well-being [7], and a meta-analysis of relationship aggression found a marginal overall effect ($d \approx 0.11$), not statistically convincing when restricted to RCTs alone ($d \approx 0.05$) [8]. None of these analyses coded a decision-control, coercion-recognition, or safe-help-seeking outcome. Tested against a harder outcome, results are mixed, not reassuring: an 8-year randomized follow-up found no divorce-rate benefit, with couples with pre-existing negative communication patterns *more* likely to divorce after the intervention [9]; couples with acute baseline risk, including physical aggression, benefited less than lower-risk couples [10].

Two adjacent literatures sharpen, not close, this gap (detailed in §4.3). Preconception medicine offers a structured checklist for a premarital contact point [11], and premarital genetic screening shows such a point can be delivered at scale but knowledge gained there does not reliably convert into behaviour: premarital sickle-cell-trait screening uptake across Africa is “suboptimal” [12], and premarital thalassaemia education raises knowledge without reliably raising uptake [13]. Decision quality is this paper’s proposed nearest measurement analogue for T-PHE’s decision-control construct, detailed in §4.3.

A second gap concerns population fit. The decision-autonomy and coercion-recognition instruments that exist were built and validated primarily in North American and European populations [16, 17, 18]; exported to Muslim-majority or Southeast/South Asian settings, cross-cultural transfer has required re-validation and has not always succeeded, e.g. a 13-item empowerment scale adapted to Arabic retained sub-optimal model fit even after revision [19], and a review of 59 studies across 22 countries found persistently poor sexual and reproductive health knowledge among Muslim women worldwide, shaped by religious and cultural framing of information access [20]. No decision-autonomy or coercion-recognition instrument in this review has been validated in a Thai population; clinical-safety evidence (§4.3) adds that cultural-competence training for health professionals repeatedly raises provider knowledge or attitudes without a corresponding patient outcome gain. T-PHE is proposed into this set of gaps: an evidence base measuring the wrong outcomes for its mechanism, outcome instruments not validated for its population, and adjacent cultural-adaptation evidence showing engagement gains do not reliably transfer into outcome gains.

1.4 Objective

To propose T-PHE as primary prevention infrastructure that increases each participant’s control over marriage-related health decisions within safeguarding and referral boundaries, and to state the programme theory in a form that can be tested and can fail. Beyond the accepted abstract, this full paper adds operational definitions,

the stop–assess–refer–record pathway, a named outcome framework, seven testable propositions, and the ethical root from which the safeguards are derived.

2 Methods

2.1 Study design

A practitioner-led, document-informed conceptual synthesis, designed to translate recurring governance and safeguarding concerns into a coherent, falsifiable programme theory. It was not a formal qualitative study and did not analyse participant-level data.

2.2 Setting and researcher positionality

The originating practice context was ARAYA Nikah, a Muslim-led social enterprise registered in Thailand, and its premarital education programme for Muslim and intercultural couples (more than 600 participants since 2024; programme reach, not study enrolment). The author is the founder and programme developer; the conflict of interest is disclosed (Declarations). Practitioner proximity provides access to organisational documents and operational dilemmas and carries a risk of confirmation bias, selective interpretation, and reputational interest. The response is not to erase practitioner knowledge but to prevent proximity from being presented as independent validation.

2.3 Data sources

1. Internal governance and safeguarding materials: role boundaries, consent protection, stop–route–record procedures, documentation and referral.
2. Premarital education materials and workflow documents.
3. Practice-level recurring concerns encountered in delivery, treated as prompts for design, not as a coded dataset or frequency estimate.
4. External frameworks and literature used for positioning: WHO violence-prevention and health-system guidance, IPV screening and referral guidance, relationship-education research, cultural humility, and selected Islamic consent sources.

2.4 Analysis

(i) Identify the decision whose protection defines programme success; (ii) identify failure modes that make apparently successful education unsafe or untrustworthy; (iii) convert ethical commitments into observable programme duties and boundaries; (iv) specify outcomes and disconfirming evidence that can be tested prospectively. A person-level scenario simulation over declared assumptions was run to rank delivery conditions; it is a sensitivity exercise, not evidence, and is reported separately in Appendix F.

2.5 Ethical framework

The synthesis used five commitments: limited knowledge, entrusted responsibility, human dignity, just boundaries, and repair. Epistemic humility is illuminated by the working sequence *faqr* → *taqwa* → *amanah*, with *shura* as the method of acting when limits are reached and *'adl* (justice) as the test of the result (Section 4.4). Each

term has a Qur'anic basis; the ordering as a T-PHE architecture is this paper's synthesis, not a model declared by the sources.

2.6 Ethical considerations and methodological limits

No systematic sampling, independent coding, saturation, participant interviews, consensus panel, control group, pre–post comparison, or causal analysis was undertaken. These limits define the next research steps; a programme theory can be useful before effectiveness testing if its derivation, boundaries, mechanism, and falsifiers are transparent.

T-PHE's safeguards assume adult participants able to consent in their own right. Where a participant is, or may be, below the age of legal capacity to marry or consent, the organisation's duty of care and any applicable mandatory-reporting obligation take precedence over R3's revocable-consent framing and R2's partner-exclusion rule; this boundary and its procedure are not yet specified and must be resolved before any pilot enrolls participants who may be minors.

2.7 Reporting guideline

No published reporting guideline currently governs a pre-effectiveness programme-theory paper of this kind; CONSORT, SPIRIT, STROBE and SRQR presuppose a conducted trial, cohort, or qualitative study, and TIDieR presupposes an already-delivered, evaluated intervention. In their absence, this paper follows an IMRAD structure and discloses its own evidence-grading practice throughout (Appendix D, Claim–evidence ledger; `CLAIMS.md` in the source repository), naming for every load-bearing statement whether it is externally sourced, conceptually derived, or proposed and untested. TIDieR will be applied to the intervention description once Phase 2 delivery is fixed.

3 Results

3.1 Definition and scope

Trustworthy Premarital Health Education (T-PHE) is a proposed governance architecture for premarital education that aims to increase each participant's control over marriage-related health decisions through five rules, a stop–assess–refer–record pathway, repairable documentation, and prospectively testable outcomes. *Trustworthy* names a condition the programme must earn through observable practice; *premarital* names the transition point, not one legal or religious form; *health education* spans physical, psychological, sexual and reproductive, relational, legal-protection, and help-seeking literacy without turning educators into clinicians, lawyers, investigators, or religious adjudicators; *infrastructure* means repeatable roles, boundaries, records, referral relationships, training, and accountability. T-PHE is placed upstream, but when it encounters existing violence or coercion its function changes to identification and safe routing; it connects to competent secondary and tertiary responses without providing them.

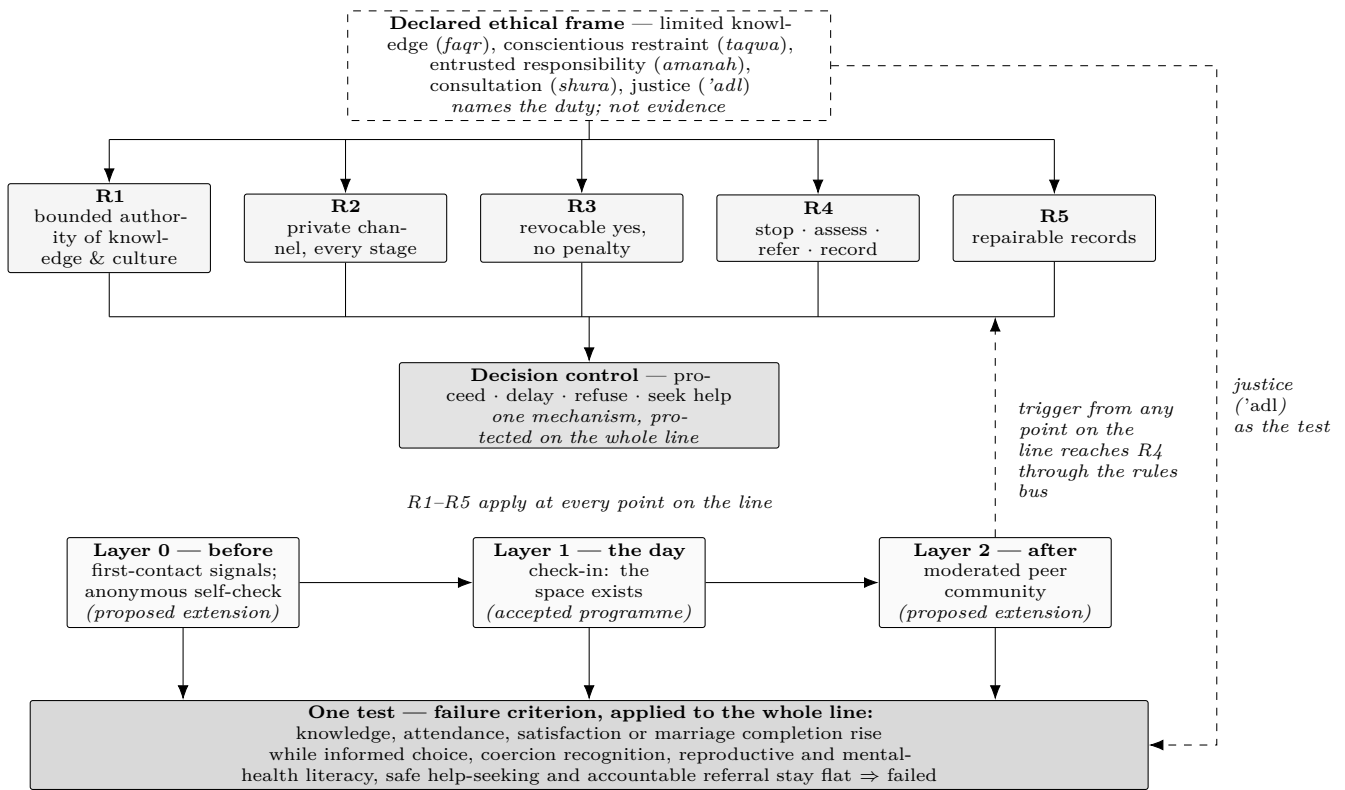


Figure 1: Programme-theory diagram of T-PHE, not a measured causal graph: no arrow represents an estimated effect. The declared ethical frame names the duty behind each rule; it is not evidence. Five rules (R1–R5) protect the single proposed mechanism, decision control; they apply at every point on one continuous timeline, of which only Layer 1 (the day) belongs to the accepted programme — Layers 0 and 2 are extensions proposed for co-design. One pre-specified test is applied to the whole line; a trigger from any point on the line can invoke R4 (stop–assess–refer–record) directly, and justice (*'adl*) is the standard the failure criterion itself is tested against.

T-PHE is *not*: a diagnostic instrument; a substitute for clinical, social, legal, emergency, or survivor-advocacy services; couple therapy or reconciliation counselling; a promise to prevent violence; a requirement that any participant leave or proceed; a universal Islamic ruling or a culturally neutral model; evidence that any organisation’s programme is effective; an autonomous AI counselling system; or a checklist detached from training, referral capacity, and governance.

From six safeguards to five rules. The accepted abstract names six safeguards that protect decision control: epistemic humility, cultural intelligibility, affected-person visibility, dignity and consent, bounded authority and referral, and repairability. Below, these are operationalised as five rules, R1–R5 (Table 1), because epistemic humility and cultural intelligibility answer the same audit question — whose authority is this claim carrying, and where does that authority end — and are therefore implemented together as R1 (bounded authority of knowledge and culture). Affected-person visibility maps to R2; dignity and consent to R3; bounded authority and referral splits across R1 and R4; repairability maps to R5. Every safeguard named in the accepted abstract survives this operationalisation; the change from six to five is a regrouping of duties into auditable rules, not a retraction of any safeguard, and the same mapping applies wherever the two vocabularies (“six safeguards”

and “five rules”) appear elsewhere in this paper.

3.2 The model as a continuous line

The programme theory does not treat the classroom day as the intervention. It treats the whole trajectory from first contact through the day of check-in to whatever follows as one governed line, on which the mechanism, five rules, and one test apply throughout. Three layers mark points on that line; only the middle layer, the day itself, is part of the accepted programme.

Layer 0 — signals from first contact (proposed extension)

Before enrolment, course-facing material can state plainly what fear, control, pressure, or unsafe participation look like, that pausing or refusing is a normal outcome, and where private help is available. An anonymous scored self-check — a form, activity, or game, usable at any point on the line — lets a person see their own result and routes without submitting a name or a narrative unless they choose to. No identity is required to complete it, and nothing is sent onward unless the person acts on it. The nearest evidence family is decision-aid research, not a self-check validated for this population: a Cochrane review of 209 decision-aid trials found consistent gains in knowledge and values-congruent choice

[14], while the closest content-matched online self-efficacy tool for women experiencing abuse, I-DECIDE, found *no* improvement in self-efficacy at 6 or 12 months in a randomised controlled trial [119, 120]. That null result is reported here, not set aside, because it is the closest outcome-domain precedent this proposed layer has. Because access to a shared or partner-monitored device is itself a disclosure risk independent of what is entered, any pilot implementation must leave no discoverable trace on a shared device (no login, no tool-specific browser or app history, a quick-exit function) and must state this limitation to users rather than implying anonymity of content is sufficient.

Layer 1 — the day as check-in

The day of delivery is reframed as a check-in at which a participant verifies in person whether the declared protections are in place: a private slot the partner cannot see or control, a professional interpreter rather than a family member, and an assessor with no stake in whether the wedding proceeds. Attendance on this day evidences that the space exists; it evidences nothing about whether the programme changed anyone's decisions, knowledge, or safety. Five rules govern conduct on the whole line and are auditable at any point, not only on the day. Table 1 states each rule's duty, one observable trace, and one falsifier.

If danger appears imminent, the routine R4 sequence is suspended in favour of the organisation's emergency protocol: immediate safety planning and contact with emergency services or the OSCC emergency line take precedence over scheduling confidential assessment. Facilitators are trained to recognise the difference and are never the final decision-maker for either pathway. A named safeguarding lead, independent of routine facilitation duty, owns supervision, escalation decisions and incident review; the assessor judges the individual case, and the safeguarding lead owns the system.

Layer 2 — moderated peer community after the course (proposed extension)

After delivery, a moderated peer space, anonymous by choice, is proposed as a route through which the same rules keep operating rather than as a service in its own right: a place to keep the self-check and the private channel alive, with a moderator trained to route a real trigger to R4 and never to mediate. It carries no promise of support beyond that routing function. The nearest evidence families are online peer-support research, where moderation quality is repeatedly identified in realist syntheses as the factor that determines whether a space helps or harms [140, 144], with mixed effectiveness reported for youth mental-health peer forums specifically [139], and technology-facilitated-abuse research, which documents how an unmoderated or poorly bounded digital channel can itself become a vector for surveillance, harassment, or coercive control [149, 150]. Neither literature was generated on a Muslim or Thai premarital population, and the pairing is by analogy, not by direct evidence

for this layer. A pilot of Layer 2 requires, at minimum, technically enforced anonymity (no real names, no shared-device login trace), a moderator-independent one-tap report channel that routes directly to R4, and a periodic anonymous safety poll of members; none of this exists yet, and Layer 2 should not be piloted without it.

One test over the whole line

The programme theory has a single, pre-specified failure criterion, applied to the mechanism, the rules, and every layer together rather than to any one point on it. Stated verbatim from the accepted abstract: T-PHE fails if it raises knowledge scores, attendance, satisfaction, or marriage completion while leaving informed choice, recognition of coercion, reproductive and mental-health literacy, safe help-seeking, and accountable referral unchanged. A polished, well-attended programme that leaves those outcome domains flat has failed as public-health infrastructure, whatever its conventional metrics show.

3.3 The declared ethical frame

The programme's own ethical language is named here as a declared frame, not as evidence: it states the duty an actor is held to, while the evidence in Table 1 and elsewhere states what is known about the corresponding construct. The two are never substituted for one another. *Faqr* (acknowledging one's own incompleteness and dependence) governs the humility required by R1. *Taqwa* (conscientious restraint in the use of knowledge and power) governs the same boundary in R1 together with the stop obligation in R4. *Amanah* (authority and knowledge held as an entrusted responsibility, to be returned to whom it belongs) governs the private channel of R2, the revocable consent of R3, and the repairable records of R5. *Shura* (consultation when a limit is reached) governs the assessor and referral step within R4. *'Adl* (justice, tested by whether the less powerful person keeps dignity, rights, voice, and a route to repair) is the standard the failure criterion applies. Table 2 pairs each term with the rule it governs, the evidence category that speaks to the underlying construct, distinct from the term itself, and an evidence tier (external evidence, analogy, open, or practitioner-derived) so a reader can see how directly each pairing is supported. None of the humility- or compassion-adjacent literature cited below was generated on a Muslim or Thai sample; it is adjacent-population evidence, not evidence for this population.

3.4 Labelling: what is accepted and what is proposed

The mechanism, the five rules, the stop rule, and the failure criterion restate the accepted abstract and carry its claim ceiling: no effectiveness claim is made anywhere in this line. **Layer 0 (signals from first contact and the anonymous self-check), Layer 2 (the moderated peer community), and the no-stake assessor are extensions proposed for co-design after the abstract's acceptance.** They are not part of the ac-

cepted programme theory, they add no outcome claim of their own, and any public text using them must say so explicitly. Where the evidence base for a proposed extension is open in either direction, that status is stated plainly rather than framed as a mild or roughly neutral uncertainty.

3.5 Testable propositions

Table 3 converts the framework into a research agenda capable of producing disconfirming evidence. These are propositions for prospective testing, not results.

3.6 Research and development pathway

Phase 0: framework clarification and evidence mapping (definitions; alignment with WHO RESPECT and clinical guidance and Thai service pathways; inventory of validated brief screens and measures; legal and data-governance review; distinguishing current ARAYA practice from proposed pilot requirements; external advisory group with conflict-of-interest management). Phase 1: co-design and content validation with survivors and advocates, Muslim and intercultural community members, Thai and non-Thai prospective participants, obstetric, reproductive-health, mental-health, social-work, substance-use, legal, and crisis-service professionals, Health Region 12, provincial Islamic committees, educators, interpreters, and data-governance expertise. Phase 1 is expected to proceed through facilitated co-design workshops together with cognitive interviewing of draft outcome instruments, with an indicative participant range of approximately 4–8 per stakeholder group, stated here as indicative rather than fixed. It would be chaired by an independent chair from outside ARAYA Nikah, and any activity involving survivors or community members would be sought through an accredited Thai research-ethics committee (for example a Health Region 12 provincial-hospital REC or an independent REC) before it begins. Phase 2: feasibility and safety pilot with pre-specified progression criteria—testing whether T-PHE can be delivered safely and consistently, not whether it reduces violence. Phase 3: prospective comparative evaluation with decision control as primary candidate outcome, provided equipoise, safety, and referral standards are protected in all arms; violence reduction is not claimed without adequate power, follow-up, and attention to disclosure bias. The failure criterion and primary outcome set will be deposited in a public registry (for example the Thai Clinical Trials Registry) before Phase 3 data collection begins. Phase 4: independent replication outside ARAYA and in different legal, service, religious, and cultural settings.

4 Discussion

4.1 Principal findings

T-PHE shifts the organising question of premarital education from content delivery to the governance of choice. Values such as respect, consent, culture, and referral become trustworthy only when translated into roles, trigger criteria, private channels, competence boundaries, records, review points, and measurable outcomes.

4.2 Novelty and standpoint

T-PHE is not proposed into an empty field. Table 4 places it alongside six adjacent bodies of practice and evidence already reviewed in §4.3, stating only what each is documented to do, measure, and not do, and what T-PHE adds on top of it. The comparison is not a claim that T-PHE outperforms any row; no effectiveness evidence exists for T-PHE itself.

Read across the rows, T-PHE's claimed novelty is not a new curriculum topic; every topic in the right-hand column already exists somewhere in the left-hand column. The novelty is a governance layer placed around premarital education, with decision control — each participant's ability to proceed, delay, or refuse, and to seek help safely — named as the mechanism, rather than knowledge gain, attendance, satisfaction, or completion. Four design choices follow directly from gaps documented above, not from this paper's own preference. First, a private channel for every person at every stage answers the finding that screening increases identification without a shown effect on referral or re-exposure [38, 39]: a channel that exists before, during, and after the day is a different exposure than a single clinical question. Second, the stop rule — joint work suspended on any trigger, resumed only after private assessment by a no-stake assessor, and never resumed where coercive control is confirmed — follows from the couples-therapy literature's own caution: the only systematic review of conjoint IPV treatment called it viable only in select situations after screening 1,733 citations to 6 eligible studies [45], and clinical literature treats distinguishing situational couple violence from coercive controlling violence as a prerequisite for any joint work at all [46, 47]. Communication and conflict training, which MRE/PREP delivers by default, is accordingly withheld rather than delivered wherever that distinction cannot yet be made. Third, counting referral at connection, not at offer, follows from the same screening literature showing identification without a demonstrated referral effect [38, 41] and from an emergency-department programme in which most screened patients could not recall receiving a resource guide despite high screening rates [172]: identification without response is not treated here as response. Fourth, repairable records and a pre-specified failure criterion follow from the recurring knowledge-to-behaviour gap this review documents twice over — premarital screening and thalassaemia education produce knowledge gains without matching uptake [12, 13], and decision-aid research shows values-congruent choice can improve while self-efficacy in the closest content-matched trial did not [14, 120] — so T-PHE pre-declares that raising knowledge, attendance, satisfaction, or marriage completion while leaving informed choice, coercion recognition, and safe help-seeking unchanged counts as failure, not success. The extensions proposed for co-design — signals from first contact, an anonymous self-check, and a moderated peer community — extend this same logic outward along the line rather than adding new content, and carry the same evidence gaps stated where each is first defined

above.

Standpoint. This proposal is made from the standpoint of a social-enterprise practitioner and citizen with a direct process position in the transaction it describes — someone who sits inside the premarital-education relationship, not above it, and who has run one such programme since 2024. It does not speak from clinical, scholarly, legal, or religious authority, and none is claimed. Those disciplines are respected on their own terms, and every construct this paper borrows from them is paired with their own evidence, tiered honestly, in §4.3 and Table 2. The organisation’s conflict of interest is disclosed in full (Declarations). The failure criterion stated above, not the author’s or the organisation’s own account of its programme, is the standard this proposal asks to be judged by.

This comparison and standpoint are offered as the basis for the co-design invitation in Phase 1 of the research and development pathway described above, not as a claim that T-PHE works.

4.3 Comparison with existing frameworks and literature

T-PHE is compared here against the literature it draws on and is answerable to, not against a systematic review’s full search strategy. Unless flagged otherwise, every citation below offers adjacent, analogous, or precedent evidence for a construct T-PHE uses — never evidence that T-PHE itself, or any of its five rules, works. The full ten-topic narrative this synthesis condenses, including the burden-of-disease case, the preconception-medicine analogue, the peer-support and technology-facilitated-abuse literatures, the assessor-independence analogy, decision-quality and digital self-assessment tool literature, language-access evidence, and the tier-one Islamic bioethics companion literature, together with the evidence map for propositions P1–P7 (Table 5), is reproduced in full, with every citation intact, in Appendix E (Extended evidence review).

T-PHE is proposed as an addition, not a replacement, to structures that already exist in this region. WHO’s RESPECT framework already names relationship-skills strengthening as one of seven population-level violence-prevention strategies, though a global review of reviews updating it found only 6 of 178 eligible reviews targeted that strategy specifically [240, 30] — a strategy named, not yet built out. Thailand’s own Health Region 12 premarital course and Malaysia’s administratively mandatory Kursus Pra-Perkahwinan Islam, examined here only through a moral-education content analysis at one Kuala Lumpur centre [242], show that a premarital contact point can already be delivered at population scale in Muslim and Thai settings. Neither is shown, in the sources available to this review, to carry a stop rule, a private channel independent of the couple, or a pre-specified failure criterion. T-PHE is offered as a governance layer that could be added to structures such as these, not as a competing curriculum (full comparison: Table 4, §4.2).

Knowledge gained at a premarital contact point

does not reliably convert into the outcome that contact point exists for. Premarital genetic screening is the closest direct precedent: premarital thalassaemia education raises knowledge ($p < 0.001$) without reliably raising uptake [13], and premarital sickle-cell-trait screening uptake across Africa pools under half of eligible couples (47.82%, 95% CI 46.53–49.11), described as “suboptimal” [12]. The relationship-education literature shows the same dissociation at larger scale: across cumulative meta-analyses, effects on relationship quality and communication skill are consistently small ($d \approx 0.03$ – 0.45) [6, 7]; the most current synthesis of a government-funded programme (Hawkins, 2022) found small effects on relationship quality ($d = 0.114$) and skills ($d = 0.132$) and non-significant effects on stability, parenting, and child well-being [7]; and a meta-analysis of relationship aggression specifically (Karantzas, 2023) found $d \approx 0.11$ overall, not statistically convincing when restricted to RCTs alone ($d \approx 0.05$) [8]. None of this literature has measured informed choice, coercion recognition, or safe help-seeking.

The same identification-without-response pattern motivates R4’s referral-at-connection rule. A Cochrane review of 13 RCTs found IPV screening increased identification (OR 2.95) but no evidence of an effect on referral (OR 2.24, 2 studies only) or on re-exposure at 3–18 months [38]. The 2025 USPSTF evidence report reviewed 35 studies ($n = 18,358$): 3 RCTs ($n = 3,759$) comparing screening with no screening found no significant reduction in IPV or other outcomes over 3–18 months [39], and the companion 2025 recommendation statement retained only a moderate-net-benefit judgement for reproductive-age women [40]. Screening finds; it does not, by itself, connect.

Recognition and behaviour dissociate in the literature closest to Layer 0’s premise. The strongest cautionary evidence for a proposed extension built around raising recognition is a 1-year cluster RCT of Coaching Boys Into Men (16 high schools, $n = 1,513$ male athletes): perpetration fell, but recognition of abusive behaviours did not change (95% CI -0.15 to 0.09), and bystander intentions were unchanged [185] — recognition and behaviour separating exactly as T-PHE’s failure criterion is built to catch. Layer 0’s proposed anonymous self-check has a direct, cautionary precedent of its own: I-DECIDE, a masked two-arm RCT ($n = 422$, 79–80% 12-month retention), found no improvement in self-efficacy at 6 or 12 months, with scores slightly lower in the intervention arm than control (mean difference 1.3, 95% CI 0.3 – 2.3 at 6 months) and no between-group difference in depressive symptoms [119, 120]. That null result is reported here, not set aside, because it is the closest outcome-domain precedent this proposed extension has.

The stop rule’s own evidence base is small and explicitly cautious. The only systematic review of couples therapy as an IPV treatment, after screening 1,733 citations, found only 6 studies met inclusion; pooled data suggested conjoint treatment “viable in select situations,” with professionals cautious about violent-retaliation risk

[45]. Survivor-voiced evidence complicates any default toward joint response: only 9 of 17 women in one qualitative sample rated couples counselling a good idea [48]. This literature supports withholding routine joint work where coercive control is confirmed; it does not demonstrate that any stop rule, including T-PHE's, improves outcomes.

Cultural-competence training repeatedly raises provider-level knowledge or attitudes without a corresponding patient-outcome gain — the pattern R1's cultural-intelligibility safeguard is built to answer, not to assume already solved. A Cochrane review of 5 RCTs (337 professionals, 8,400 patients) rated the evidence low-quality, with some improvement in involvement or understanding but no reliable clinical-outcome effect [55]; a narrower review found no significant patient-outcome improvement in any of the studies it identified [56]; and Indigenous cultural-safety training across four high-income countries found outcomes overwhelmingly provider-level, with weak clinical-outcome evidence [57].

Religiosity is not a uniform protective factor, which is why the ethical frame (§3.3) is presented as a declared duty rather than as evidence. A 67-study systematic review and meta-analysis found religiosity and spirituality protective for general interpersonal aggression but *not* specifically for domestic violence [215], complicating any simple “faith protects” reading and reinforcing why Table 2 pairs each ethical term with a separately evidenced construct rather than treating the term itself as a finding.

Direct Thai evidence remains sparse and geographically uneven, and none of it evaluates a premarital programme. A four-region national survey ($n = 2,462$ married or cohabiting women) found roughly 15% lifetime psychological, physical, or sexual IPV [72]; among 700 Muslim adolescents on the southern border, 9% were sexually experienced, with condom non-use, STI, and repeat pregnancy concentrated in that subgroup [76]; and among 360 Muslim army conscripts in the three deep southern provinces, 78.7% showed poor HIV/STI-transmission knowledge [77]. No study identified in this review evaluates a premarital-education programme, of any design, with a Thai Muslim or Thai intercultural population.

Read together, this literature does two things and stops short of a third. It makes a premarital contact point plausible to build and deliver at population scale (preconception medicine, premarital genetic screening, and Thailand's and Malaysia's own existing courses); it repeatedly shows that knowledge, attendance, satisfaction, and provider-level attitude gains do not reliably convert into the behavioural, referral, or patient-outcome change a programme is built to produce (screening, relationship education, and cultural-competence training alike). It does not show that any premarital governance layer, of this or any design, changes informed choice, coercion recognition, or safe help-seeking — because no such test has yet been run. T-PHE is offered as a falsifiable synthesis awaiting that test, never as evidence that premarital education of this design improves, prevents,

or reduces any outcome. The full evidence base this synthesis draws from, organised by proposition (P1–P7) and by topic, is in Appendix E.

4.4 Ethical root: *faqr*, *taqwa*, *amanah*, and justice

The framework does not treat culture as a barrier to be removed. Its epistemic-humility safeguard rests on a working sequence: *faqr* makes us aware of our own incompleteness and dependence on God (Qur'an 35:15); *taqwa* is God-conscious restraint over one's knowledge, speech, judgement, and use of power (Qur'an 59:18); *amanah* frames authority and knowledge as an entrusted responsibility to be rendered to those to whom it belongs, placed by the Qur'an beside just judgement (4:58). *Faqr*'s classical sense is existential neediness before God; this paper narrows it to the epistemic case (what one does not know) as one instance of that neediness, not as a redefinition of the term. *Shura* is the method of acting when limits are reached (42:38); it accompanies the sequence and does not replace *taqwa* within it. *'Adl* is the test: not whether the person holding authority meant well, but whether the affected person retains dignity, rights, voice, and a real route to correction.

Applied to the family, in the author's formulation: *faqr* makes us aware of our own incompleteness and our dependence on God; because no one is complete, the members of a family must depend on and be responsible to one another, with *taqwa* as the governing restraint, and with *amanah* made visible through the steady, dependable fulfilment of duties that never violate one another's rights. Mutual dependence is the reason a family needs safeguards; *taqwa* is why authority within it must be restrained; *amanah* is why fulfilling one's role and respecting the other's rights are the same act rather than competing ones. Humility here is not passivity: a person may fulfil duties, ask for help, challenge misuse of authority, and claim rights with humility, and a teacher who admits uncertainty must still act when safety procedures require it. This is compatible with cultural humility in health education [58]. The sequence is this paper's conceptual synthesis; the terms are Qur'anic, the architecture is not a model declared by the sources, and full jurisprudential interpretation requires qualified scholarship. Where Table 2 marks a term's evidence pairing Open, the rule it governs (R1 for *taqwa*, R2/R3/R5 for *amanah*) is supported only by the rule's own falsifier and observable trace in Table 1, not by external evidence for the ethical term itself.

4.5 Implications for practice and policy

Without a failure criterion, programmes can absorb safeguarding language while retaining continuation-oriented incentives. If knowledge and satisfaction rise while a person remains unable to pause, refuse, recognise coercion, or reach qualified help, the mechanism has not been achieved. Infrastructure should be judged by what it enables under pressure, not by what it teaches under ideal conditions. A scenario simulation over declared assumptions (Appendix F) ranked pre-arranged assessor, room and interpreter, a facilitator-training gate, a

narrow trained trigger list and a follow-up slot as the delivery conditions most likely to decide whether the stop rule can be executed; it produces no evidence about outcomes.

4.6 Institutional position

Why does a social enterprise, rather than a clinical, academic, or religious institution, convene this synthesis? What follows is a declared position, not an evidentiary claim, and it rests on process, not authority. ARAYA Nikah occupies what this paper calls the process position: it is present with a couple across the weeks before a wedding, controls the sequence in which content, documentation, and referral are offered, and observes what actually happens at the point where consent, authority, and help-seeking are organised. That sustained proximity is not ordinarily available to a single clinic visit, a one-off curriculum audit, or a legal review conducted after the fact.

The organisation is registered, per the organisation's own governance records, under Thailand's Social Enterprise Promotion Act B.E. 2562, which binds it in law to a declared social objective and limits the distribution of profit. That legal binding, and no broader legal claim, is what is asserted here; the Act's other provisions, and any claim about prenuptial law or domestic-violence law reform, are outside this paper's competence and are not made. Because the organisation's continuation does not depend on completing marriages, it can afford to declare a failure criterion that treats marriage completion as irrelevant to success — a criterion a fee-for-service or completion-incentivised provider would find harder to state without undermining its own model.

This position carries no authority of its own, and none is claimed. It offers depth in one relationship, repeated many times, not population coverage; a clinical, academic, or religious body with broader reach and different obligations is not replaced by it and is not asked to defer to it. Its legitimacy is borrowed, not earned: it stands until survivors, advocates, religious and community leaders, and independent reviewers examine the procedures described in this paper and say plainly whether they hold up. Being bound by law to a social objective changes what the organisation's incentives are re-bound to; it does not remove incentive, interest, or the founder's disclosed conflict (Declarations), and none of the above is offered as a substitute for that independent review.

4.7 Implications for research: transferability and next steps

T-PHE originated in a Muslim-led Thai practice environment but addresses a general problem. Transferability is an empirical question. Adaptation may change language, examples, religious sources, legal procedures, service partners, facilitators, and settings; it must preserve the invariants: the participant, not marriage completion, is the unit of protection; usable decision control is the mechanism; each person has a safe private route; culture and religion do not override consent or safety; educator authority is bounded; fear, violence, or coercive con-

trol changes the course of education; qualified referral takes priority beyond competence; decisions and records can be reviewed and corrected; conventional outcomes cannot substitute for safeguarding outcomes.

4.8 Artificial intelligence: boundary only

AI is not part of the T-PHE mechanism and is not an evaluated intervention (distinct from the manuscript-drafting disclosure in Declarations). Any future exploration is limited to facilitator-reviewed drafting of teaching materials or fictional scenarios, with no AI counselling, no autonomous safety decisions, no automated classification of violence or coercion, and human review for accuracy, cultural meaning, safety, and bias. No participant data are entered into any AI system; the anonymous self-check (Results, Layer 0), if used, is scored without AI.

4.9 Strengths and limitations

Strengths: the programme theory is stated with a pre-specified failure criterion, named outcome domains, and seven propositions with falsifiers, before any effectiveness testing; the claim ceiling is explicit; the conflict of interest is disclosed. Limitations: (1) Derived from one founder-led organisation. (2) Not a formal qualitative study: no systematic sampling, independent coding, or participant validation. (3) No effectiveness or safety testing; the mechanism, safeguards, measures, and pathway may prove incomplete, burdensome, or harmful. (4) Grounded in Muslim and intercultural couples in Thailand; not representative of all Muslim communities, Thai settings, or non-Muslim contexts. (5) Identifies a governance problem without establishing that existing programmes lack comparable safeguards; direct claims about any programme require its complete procedures to be examined. (6) Decision control is not a validated measure and may overlap with autonomy, self-efficacy, informed choice, safety, and service access. (7) Referral capacity is context-dependent. (8) Founder authorship creates conflict of interest; independent co-design, external review, and replication are necessary.

4.10 Invitation to co-design

This framework is offered at the point where practice-derived synthesis must stop and shared testing must begin. ARAYA Nikah does not propose to validate T-PHE alone, and does not claim the authority to decide what "trustworthy" means for communities it does not represent. What the organisation can offer into a Phase 1 co-design process (§3.6, "Research and development pathway") is what this paper has already made explicit and auditable: its governance and safeguarding procedures, its stop-assess-refer-record pathway, its documentation practice, its practice context of 600+ participants offered here as demand for this work, not evidence that it succeeds, and a pre-specified failure criterion against which its own work is willing to be judged — T-PHE fails if knowledge, attendance, or satisfaction rises while informed choice and accountable referral do not.

Co-design under that criterion needs partners this

paper cannot substitute for. Survivors and advocates are needed to test whether the private channel and the stop rule protect rather than perform protection. Health Region 12 and its existing premarital services are needed as the clinical and public-health infrastructure this framework is designed to add governance to, not replace. Provincial Islamic committees are needed because premarital education in the deep South already has standing and reach this framework does not. This framework does not yet know, and does not assert, whether OSCC and equivalent referral services are reachable within a safe timeframe across all delivery sites, including after-hours and in Malay/Yawi-medium settings, or how a state referral pathway is meant to interact with a participant's existing recourse to family or religious dispute resolution; these are named here as open questions for Phase 1, not as solved problems. Referral services, including One-Stop Crisis Centres (OSCC) and other designated multidisciplinary services, are needed to confirm that the pathways named here are real, timely, and accessible where couples actually live. TIMA/FIMA members are needed as the clinical judgement this synthesis was never positioned to supply on its own.

Phase 1 would be chaired independently of ARAYA Nikah, with decision authority in a steering group on which the organisation holds one seat and no veto. Materials co-developed in Phase 1 would be held by the co-design consortium under a Creative Commons Attribution (CC BY 4.0) licence, not by the company. Partners take on reputational exposure to a framework that has not yet been tested; the failure criterion and the right to withdraw at any phase are offered as the protection against that exposure.

The organisation's contribution ends where these partners' expertise begins. What is asked in return is not endorsement but scrutiny: a Phase 1 co-design process free to keep, revise, or reject any element of this synthesis, tested against the failure criterion stated above and in the accepted abstract. Nothing here proceeds to scale before feasibility and safety testing (Phase 2) confirm that T-PHE can be delivered safely and consistently, before any claim of effectiveness is made.

5 Conclusion

Premarital health education is a consequential decision environment. When it teaches couples but cannot protect the individual participant, stop unsafe joint work, refer beyond educator competence, or correct an earlier decision, it remains incomplete as public-health infrastructure. T-PHE proposes one mechanism—decision control—protected by six safeguards, operationalised as five rules (R1–R5), and a stop rule under which routine couple education yields to confidential assessment, survivor-centred support, and qualified referral. The framework is not yet validated; its value will depend on whether co-design and empirical testing show that it improves usable choice and accountable routing without creating new harms. Judge premarital health education not by how many marriages it completes, but by how safely people can choose. That judgement is not the or-

ganisation's to make alone: it is offered to TIMA/FIMA, Health Region 12, provincial Islamic committees, referral services, survivors, and advocates as an invitation to co-design and test this framework together, under the failure criterion stated above, before any claim of effectiveness is made.

Declarations

Conflict of interest. The author is the founder and programme developer of ARAYA Nikah Social Enterprise Co., Ltd., the organisation whose procedures informed this synthesis, and has a reputational and commercial interest in the programme described. This conflict is disclosed in every public version of the work. No grant or award funded this work.

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Ethics. This conceptual paper involved no participant research, no participant data, and no identifiable information. Any co-design, cognitive interviewing, pilot data collection, or evaluation will undergo ethics review before it begins.

Data availability. No participant dataset was generated or analysed. The scenario-simulation code and its declared parameters (Appendix F) are available from the corresponding author on request.

Use of AI. AI tools assisted drafting and editing; the author verified all content and is accountable for it. No participant data are entered into any AI system; the anonymous self-check (Results, Layer 0), if used, is scored without AI.

Author contributions. YL conceived the framework, conducted the synthesis, and wrote the manuscript; there are no further contributors.

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Disclaimer. This paper is a theoretical framework. It is not medical, legal, or religious advice, not a diagnostic instrument, and not a substitute for qualified clinical, legal, or survivor-support services. Laws governing health advice, safeguarding, data protection, and marriage vary by jurisdiction; institutional, educational, or public use requires legal and professional review.

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Table 1: The five rules (R1–R5), one observable trace and one falsifier each. Programme theory; none of these traces has yet been measured against an outcome.

	Duty	Observable trace	Falsifier
R1	Bounded authority of knowledge and culture: state what is known, uncertain, or needs qualified advice; culture or religion never closes a safety or consent question.	Uncertainty and role boundaries stated in audited sessions; referral for out-of-scope questions.	A session in which a cultural or religious claim is used to end a safety or consent question rather than to refer it.
R2	A private channel for every person, at every stage, that the partner cannot see or control; interpretation is provided by a trained, confidentiality-bound professional interpreter with no prior relationship with either participant's family, never by a family member.	A routinely offered, safely designed private slot; uptake logged without content.	A person who wanted a private slot did not receive one, or a family member interpreted a safety-relevant exchange.
R3	Revocable yes: pause, delay, refusal, or non-completion is a normal outcome, without penalty, at any point on the line.	A pause, delay, or refusal recorded without an accompanying penalty or exclusion.	A participant who paused, delayed, or refused was excluded, penalised, or pressured to reverse the decision.
R4	Stop–assess–refer–record: a trigger from any point on the line (self-check, private slot, peer community, direct observation) stops joint work, moves to private assessment by a qualified, competence-bounded assessor, refers to a qualified multidisciplinary service such as Thailand's One-Stop Crisis Centre (OSCC) or an equivalent designated service, counts referral at connection, and records the sequence; joint work does not resume where coercive control is confirmed. No couple mediation continues once coercive control is confirmed. <i>Proposed extension, not yet accepted:</i> that assessor should additionally have no stake in whether the wedding proceeds and no prior personal, family or congregational relationship with either participant that a reasonable observer would see as compromising independence, drawn where possible from outside the couple's own community network (analogy evidence only; [157, 158, 161, 162]). If a qualified assessor cannot be reached within a pre-specified maximum interval, or assessment is inconclusive, the default is continued stop, not resumption; the case is escalated to a named safeguarding lead, not closed by facilitator judgement.	Time from trigger to a qualified private assessment; referral offered, connected, and recorded.	Joint work continued, or resumed, after a trigger without a documented safe assessment; a case resumed or closed by facilitator judgement because an assessor was unavailable, without safeguarding-lead sign-off.
R5	Repairable records: every entry has a named owner, a review date, and is corrected when new information arrives.	A review completed on or before its scheduled date, with a dated correction where warranted.	A record that is never reviewed, or a correction that overwrites the original entry rather than appending to it.

Table 2: The declared ethical frame: term, gloss, governed rule, evidence category paired with it, and the evidence tier. The term names a duty; the evidence category speaks to the construct, not to the term. External-evidence and analogy citations are from adjacent, non-Thai, non-Muslim, non-premarital populations unless stated otherwise. R4’s no-stake-assessor clause, governed here by *Shura*, is a proposed extension, not yet part of the accepted programme (Table 1).

Term	Plain-language gloss	Governs	Evidence category paired with it	Evidence tier
Faqr	Acknowledging one’s own incompleteness and dependence.	R1	Intellectual and relational humility, self-compassion [204, 206, 207, 208]; declared-frame companion literature in Islamic medical ethics [220, 222].	External evidence (analogy); no Muslim or Thai samples for the humility/compassion constructs.
Taqwa	Conscientious restraint in the use of knowledge, speech, judgement, and power.	R1, R4	Self-regulation and provider-boundary and referral literature.	Open: no evidence pairing identified for this construct at this population.
Amanah	Authority and knowledge held as an entrusted responsibility, returned to whom it belongs.	R2, R3, R5	Consent, autonomy, and accountability literature.	Open: no evidence pairing identified for this construct at this population.
Shura	Consultation when a limit is reached.	R4 (assessor and referral step)	Independent-advocate and dual-relationship literature [157, 158, 161, 162].	Analogy: transplant-advocate and dual-relationship ethics, not domain-matched.
’Adl	Justice: whether the less powerful person keeps dignity, rights, voice, and a route to repair.	The failure criterion	Outcome-domain and safeguarding literature the failure criterion is built from (see discussion §4.3).	Practitioner-derived: the criterion itself is authored for this paper, not drawn from a single cited instrument.

Table 3: Testable propositions, competing explanations, and evidence that would weaken or falsify each.

	Proposition	Competing explanation	Weakening / falsifying evidence
P1	A governance layer improves decision control beyond content-only education	Knowledge or facilitator warmth explains any gain	No added gain in decision control after controlling for knowledge and facilitator effects
P2	A routine private channel makes materially relevant concerns more visible	Private contact adds burden without information	No increase in safe voice or appropriate routing, with more distress, retaliation, or privacy risk
P3	R1's cultural-intelligibility component improves comprehension and trust only when culture or religion cannot override consent or safety	Cultural tailoring alone suffices	Tailoring improves satisfaction but leaves consent pressure or unsafe authority unchanged
P4	A stop rule prevents routine joint education from worsening risk when fear or coercion appears	Skilled facilitators can manage all concerns jointly	The stop rule causes more net harm than governed alternatives, or does not reduce unsafe joint exposure
P5	Referral quality depends on timeliness, accessibility, and follow-through, not offer alone	Information is enough	Connection and safety do not vary by latency, accessibility, or follow-through
P6	Conventional programme success can coexist with T-PHE failure	Conventional outcomes represent benefit	Conventional outcomes reliably predict decision control, help-seeking, and referral fidelity
P7	R5 (repairable records) is necessary because relevant information emerges over time	One-time assessment suffices	Review and correction add no safety or accuracy benefit and introduce more harm or burden

Table 4: What existing frameworks and programmes do, measure, and do not do, against what T-PHE proposes to add. Descriptive only; several rows are not examined here at manual/curriculum level and are marked accordingly. No effectiveness comparison is implied.

		What it does	What it measures	What it does not do (as documented)	What T-PHE adds
WHO SPECT	RE-	Names seven population-level prevention strategies for violence against women, synthesised across a global review of reviews [240, 30].	Framework-level: which strategy types have supporting reviews, not a single programme's outcomes.	Only 6/178 reviews updating it targeted relationship-skills strengthening specifically [30]; it does not specify a premarital governance layer, a stop rule, or an assessor with no stake in the outcome.	Places one governance layer, with a named mechanism and stop rule, inside RESPECT's relationship-skills strategy, at the marriage transition specifically.
Thai premarital course, southern provinces / Health Region 12	Region	Delivers premarital content within the government health-region system in Thailand's southern border provinces. <i>Not examined here at manual level.</i>	Not established by any PubMed-indexed record in this review; no Thai premarital-education outcome study was located.	Not examined here — this paper does not state whether it has, or lacks, a stop-and-refer rule, because its manual has not been reviewed.	A published, auditable rule set (R1–R5) and failure criterion that any programme, including this one, can be checked against.
Malaysian Islamic pre-marriage course (Kursus Pra-Perkahwinan Islam)	pre-marriage	Reported, not independently verified by this review, to be administratively linked to marriage registration for Muslim couples in Malaysia; content analysed through a moral-education lens at one Kuala Lumpur centre [242] (non-PubMed, context only).	Moral-education framing of curriculum content at one centre; not an outcome trial.	Not shown, in the cited source, to measure decision control, coercion recognition, or safe help-seeking.	A decision-control mechanism and stop rule layered onto administratively mandatory delivery, not assumed present in it.
Relationship/marriage education (MRE, PREP)	edu-	Structured curricula teaching communication and conflict skills, evaluated across cumulative meta-analyses [6, 7, 241].	Relationship quality, communication skill, and (more recently) mental health and coparenting; effects small ($d \approx 0.03$ – 0.45) [6, 7].	Does not measure informed choice, coercion recognition, or safe help-seeking; higher-conflict couples showed a harmful-moderation pattern in one 8-year RCT [9].	A stop rule that withholds joint communication/conflict training precisely where coercive control is suspected, rather than delivering it universally.
IPV screening in health settings		Structured clinical inquiry about intimate-partner violence, reviewed in a Cochrane review and the 2025 USPSTF evidence report [38, 39, 40].	Identification (OR 2.95 for increased detection [38]); instrument sensitivity 26–87% [39].	No evidence of an effect on referral (OR 2.24, 2 studies) or re-exposure at 3–18 months [38, 39]; WHO guidance conditions inquiry on a functioning referral system already existing [43, 44].	Referral counted at the point of connection, not at the point of offer, plus a record that is reviewed and corrected rather than closed at first entry.
Digital safety decision aids (I-DECIDE, myPlan)	safety aids	Scored, self-directed online tools that let a person assess risk and see safety options privately [119, 120, 123, 124].	Self-efficacy, reproductive coercion, depressive symptoms; the largest RCT found no self-efficacy gain at 6 or 12 months [120].	Not built for, or tested at, the premarital transition; not linked to an in-person stop–assess–refer pathway.	An anonymous self-check offered as one entry point on a continuous line that also includes an in-person check-in and a named stop rule, not a standalone tool.
Faith-integrated health interventions (mosque-based RCTs)	in-	Structured health curricula delivered by imams or lay religious facilitators, tested in cluster-randomised trials [225, 226].	Clinical and psychological outcomes: 12-month type 2 diabetes incidence (9.8% vs 17.1%, aHR 0.75) [225]; PTSD, depression and well-being effect sizes [226].	Neither trial tests premarital content, decision control, or coercion recognition; delivery-channel precedent only.	Uses the same trusted-channel logic for premarital delivery, paired with a bounded-authority rule (R1) that keeps religious framing from closing a safety question.

A Construct dictionary

Construct	Working definition	Not equivalent to	Observable trace	Status	
Decision control	Usable, timely, safe capacity to proceed, pause, delay, refuse, withdraw, revise, or seek help	Satisfaction; knowledge alone; relationship continuation	Options explained, private voice, enacted pause, safe help route	Proposed	primary mechanism; requires validation
Epistemic humility	Admission of limits, source transparency, consultation, accountable authority	Indecision; passivity; silence	Uncertainty marked, roles stated, out-of-scope consultation	Conceptually	derived
Cultural intelligibility	Meaningful guidance within culture without unchecked cultural authority	Cultural competence as mastery; cultural exemption from safety	Language adaptation, source labelling, comprehension and enactability check	Conceptually	derived
Affected-person visibility	Direct, safe visibility of each person who bears consequences	Couple treated as one voice; forced disclosure	Routine private route, safe contact, privacy controls	Conceptually	derived
Dignity and consent	Ongoing respect for each person's decision and refusal	One-time signature; family or programme permission	Consent revisited; pause or refusal enacted without penalty	Conceptually	derived
Bounded authority and referral	Role limits connected to competent, timely service pathways	Abandonment; referral leaflet alone	Stop criteria, service map, warm referral, follow-through	Conceptually	derived
Repairability	Review and correction when new information appears	Erasure of audit trail; automatic restart	Review points, corrected record, incident learning	Conceptually	derived
Trustworthiness	Earned property of a governed programme whose observable practice protects choice and safety	Branding; participant trust alone	Safeguard fidelity, harms, accountable referral, failure criterion	Higher-order	evaluative construct

B Minimum viable T-PHE pathway

Before delivery. Define scope and exclusions; verify referral services and emergency routes; train and assess facilitators; name a safeguarding lead, independent of routine facilitation duty, who owns supervision, escalation decisions and incident review; establish supervision and escalation; prepare language access and non-Thai pathways; separate administrative and proposed safeguarding record systems, with access restricted to the named safeguarding lead and assessor, a defined retention period, and a documented protocol for third-party requests (including from a spouse, family member or religious authority) that defaults to non-disclosure absent a legal requirement; explain consent, privacy, and limits of confidentiality.

During routine education. Mark source and uncertainty; make content culturally intelligible; offer each person a safely designed private voice; reconfirm the right to pause, delay, refuse, withdraw, or seek help; observe for fear, threats, control, unsafe participation, or out-of-scope needs.

When triggered. (0) If danger appears imminent, the routine sequence below is suspended in favour of the organisation's emergency protocol: immediate safety planning and contact with emergency services or the OSCC emergency line take precedence over scheduling confidential assessment; facilitators are trained to recognise the difference and are never the final decision-maker for either pathway. (1) Stop routine joint work. (2) Protect immediate privacy and safety. (3) Arrange confidential assessment within defined competence. (4) Refer to qualified services. (5) Record the minimum necessary reason, action, referral, and safe follow-up. (6) Review; do not restart joint work without documented safe assessment. (7) Do not resume couple mediation where coercive control is confirmed.

After delivery. Review referral connection and barriers where safe and consented; correct records or decisions when new information emerges; audit fidelity and harms; use incidents and complaints for system repair.

C Candidate outcome and measurement matrix

Domain	Candidate indicator	Data source	Timing	Safety note
Decision control	Adapted decision-quality/ shared-decision-making instrument (see Appendix E, “Decision quality, autonomy measurement...”), context-adapted per the MRC ADAPT guidance [53]; no instrument yet validated in a Thai or Thai Muslim population.	Individual confidential survey	Baseline, post, follow-up	Avoid partner-visible completion
Option awareness	Correct identification of proceed/pause/delay/ refuse/help options	Knowledge task	Post	Does not prove enactability
Private voice	Safely offered; perceived freedom to speak	Process log, confidential survey	During, post	Do not record disclosure content unnecessarily
Coercion recognition	Scenario-based recognition	Individual assessment	Baseline, post	Trauma-informed fictional scenarios
Mental-health literacy	Recognition and help-route knowledge	Survey, scenario	Baseline, post	Not diagnosis
Reproductive-health lit- eracy	Consent, contraception, reproductive coercion, care pathways	Survey, scenario	Baseline, post	Protect sexual-health privacy
Legal-protection liter- acy	Status verification, documentation, available help	Survey, task	Baseline, post	Jurisdiction-specific
Stop-rule fidelity	Correct facilitator response in simulation or audit	Observation	Training, deliv- ery	Independent review preferred
Referral fidelity	Offer, consent, connection, latency, failed route	Safeguarding log	Trigger, follow- up	Minimum necessary data
Cultural intelligibility	Comprehension and relevance across groups	Survey, interview	Post	Analyse differential outcomes
Repairability	Appropriate review or correction completed	Audit	Ongoing	Preserve protected audit trail
Harms	Privacy breach, retaliation concern, distress, unsafe referral, inappropriate restart	Incident system, safe follow-up	Immediate, ongo- ing	Independent safety governance
Conventional outcomes	Attendance, satisfaction, completion, knowledge	Programme records	Post	Secondary; never sufficient alone

D Claim–evidence ledger

Claim	Evidence type	Current support	Allowed wording	Not yet allowed
IPV produces major health burden	Externally observed or modelled	WHO and GBD sources	“IPV is a major public-health problem with physical, mental, sexual, and reproductive consequences.”	Local causal estimates not measured
Thailand recorded an average 42 affected persons per day in 2024	Official administrative report	DWF page	“Recorded cases averaged 42 persons per day across children, women, or family violence.”	“Thai prevalence is 42/day”; Muslim-specific claim
ARAYA reached 600+ participants since 2024	Organisation-reported practice context	Internal programme record	“Programme reach exceeded 600 participants.”	Effectiveness, representativeness, harm reduction
Decision control is the T-PHE mechanism	Conceptual derivation	This synthesis	“T-PHE proposes decision control as its mechanism.”	“Decision control has been validated as the mechanism.”
Six safeguards protect decision control	Conceptual derivation	This synthesis and practice-source logic	“Six safeguards are proposed.”	“The safeguards are proven effective.”
Stop rule is safer when fear or coercion appears	Normative design and external clinical safety logic	WHO guidance; bounded couple-intervention literature	“T-PHE requires routine joint work to stop pending qualified assessment.”	Comparative safety superiority before testing
T-PHE improves outcomes	Open empirical question	None	“T-PHE is designed to improve. . .” / “should be tested for. . .”	“T-PHE improves,” “prevents,” “reduces”
Framework transfers beyond ARAYA	Open empirical question	Conceptual relevance only	“Potentially adaptable; requires local testing.”	Universal applicability

Note. The six safeguards named in the row above are operationalised in Results as five rules, R1–R5 (§3.1, Table 1): epistemic humility and cultural intelligibility are jointly implemented as R1; affected-person visibility maps to R2; dignity and consent to R3; bounded authority and referral splits across R1 and R4; reparability maps to R5. Every safeguard survives; “six safeguards” and “five rules” name the same commitments at two levels of description throughout this paper.

E Extended evidence review

This appendix carries the full literature narrative and the evidence map for propositions P1–P7 that Discussion §4.3 (“Comparison with existing frameworks and literature”) synthesises in condensed form. It is reproduced in full so that every citation can be checked in context.

Unless flagged otherwise, every citation below offers adjacent, analogous, or precedent evidence for a construct T-PHE uses — never evidence that T-PHE itself, or any of its five rules, works; this single sentence replaces a per-subsection restatement of that caveat.

E.1 Burden

The population-level case for treating the marriage transition as a safeguard point rests on burden-of-disease and surveillance data, not on any evaluation of premarital education. §1.3 established the burden case (GBD 2023 [1]; WHO/LSHTM [2]: 27% lifetime prevalence, UI 23–31%, ever-partnered women 15–49, substantial exposure already in the 15–19 and 19–24 bands). A companion burden-of-proof analysis found a moderate association with major depressive disorder and maternal abortion/miscarriage (its largest), and only weak associations with HIV, anxiety and self-harm [21]. A Lancet Psychiatry Commission situates these estimates within a wider call for mental-health-service investment [22]; a Lancet Public Health review identified only 30 unique interventions from 16 countries addressing IPV and/or violence against children [23] — sparse relative to the burden. These are surveillance/modelling estimates, not evidence any intervention changes them.

E.2 Primary prevention frameworks, what works, and recognition

The strongest experimental evidence that structured relationship curricula can reduce IPV comes from community-mobilization and couples-curriculum trials outside the premarital-education literature. SASA! was associated with lower social acceptance of IPV (aRR 0.54) and a physical-IPV effect whose CI crossed 1 (aRR 0.48) [24], with norms change — not knowledge or attendance — explaining 70–95% of the mediated effect [25]. Indashyikirwa in Rwanda found a large reduction in women’s reports of IPV (aRR 0.44) and men’s reports of perpetration (aRR 0.54) [26], while a faith-based trial in Nigeria reported a 61% reduction in odds of any reported IPV [27] — the clearest faith-setting positive result in this review. Set against these are equally rigorous null results: a scaled adaptation in rural Rwanda found no effect on any primary outcome [28], and a faith-leader caregiving trial in Kenya found no effect on its declared primary outcome while showing secondary IPV effects [29] — the outcome-substitution pattern T-PHE’s failure criterion is designed to flag. Only 6/178 (3%) reviews updating the WHO RESPECT framework targeted relationship-skills strengthening specifically [30], and a scoping review of implementation science found fidelity, acceptability and feasibility the most-studied outcomes, with scale, sustainability and survivor-centred practice minimally addressed [31] — a field-level gap consistent with treating T-PHE as a programme theory rather than advancing an effectiveness claim at this stage.

Psychological inoculation — drawn from misinformation research, not interpersonal coercion, and used here only as an analogy — is the closest experimental literature to Layer 0 (a proposed extension)’s “awareness as protection” claim. The largest field experiment (7 preregistered studies, 6 RCTs, $n = 6,464$, plus a YouTube field study $n = 22,632$) found inoculation videos improved recognition of manipulation techniques and sharing-decision quality [177] (effect sizes not reported in the abstract), extended to vaccine misinformation [178], election misinformation across 12 EU nations [179], a gamified format [180], real-world cross-protection against untrained techniques [181], and methodological work isolating technique from testing effects [182] — none of it about intimate-partner coercion. The strongest cautionary evidence comes from a 1-year cluster RCT of Coaching Boys Into Men (16 high schools, $n = 1,513$ male athletes): perpetration fell, but recognition of abusive behaviours did not change (95% CI -0.15 to 0.09) and bystander intentions were unchanged [185] — recognition and behaviour dissociating exactly as the failure criterion is built to catch. A Cochrane protocol for dating-violence prevention

has not yet reported outcomes [183]; a pilot questions whether attitudes proxy behaviour [184]; further design and implementation papers report no recognition outcomes yet [186, 187]; a non-randomised bystander study found effects on risk recognition and perceived self-efficacy [188]. Qualitatively, women describe difficulty naming gaslighting despite describing its effects [189]; being silenced through negative responses to help-seeking, and loneliness, form part of the pathway to later intimate-partner violence and abuse in young adults [190]; disclosure barriers are documented among sexual and gender-minority survivors [191]; two studies of South and Southeast Asian women describe culturally specific understandings and barriers relevant to naming abuse [195, 196] (measurement tools not yet applied here: an internalised-stigma scale [192], Thai/general health-literacy instruments [193, 194]). This literature makes Layer 0's premise plausible and testable, not established; recognition and help-seeking are shown here to be separable outcomes.

E.3 Preconception medicine and premarital screening as a proposed analogue

Preconception medicine treats the period before conception/marriage as a distinct clinical window: the FIGO Preconception Checklist frames structured health-assessment elements for it [11]; couple-based preconception-care programmes have been reviewed across 14 studies in 10 countries, mostly high-income, heterogeneous designs [32]. Premarital genetic screening is the closest direct precedent for a premarital contact point at population scale (roughly 30–40% of the Thai population carries a thalassaemia gene [117], motivating national couple-screening programmes): premarital sickle-cell-trait screening uptake across Africa pools under half of eligible couples (47.82%, 95% CI 46.53–49.11), “suboptimal” [12]; premarital thalassaemia education produces consistent knowledge gains ($p < 0.001$) but inconsistent screening-uptake behaviour [13] — the same knowledge/behaviour gap recurring below.

E.4 Premarital/relationship education outcomes and the knowledge-to-well-being gap

Across three cumulative meta-analyses spanning 1997–2022, the MRE evidence base has consistently measured relationship quality and communication skill [6, 33], and more recently mental health and coparenting [7], with effects real but small ($d \approx 0.03$ – 0.45 [6, 7]) and authors flagging thin racial/ethnic diversity [6] and rarely measured policy-relevant outcomes [7]; a third meta-analysis in this lineage examined programmatic moderators such as dosage and communication emphasis [33]. Individual RCTs complicate rather than resolve this: an 8-year follow-up found no divorce benefit and a harmful-moderation pattern for higher-conflict couples [9]; a 3-year RCT found couples with acute baseline risk benefited less [10]; a later meta-analysis of relationship aggression (31 studies, $n = 25,527$) found $d \approx 0.11$ overall, not statistically convincing when restricted to RCTs ($d \approx 0.05$) [8]. Newer, smaller trials sharpen the specific knowledge-to-well-being gap this paper labels its “middle of the bridge”: a quasi-experimental study among engaged couples in Babol, Iran, found booklet- versus video-based premarital education improved reproductive-health-literacy access, evaluation and decision-making dimensions, but not reading-and-understanding [34]; a 2026 study of reproductive/sexual-health education in premarital counselling classes in Rasht, Iran, found knowledge and attitudes improved at 3 months but reported no clinical outcomes [35]; and a meta-analysis of online relationship-education programmes (study populations not characterised in the abstract) found they improved satisfaction, communication, confidence and individual-level anxiety/depressive symptoms [36]. At the far end of this pathway, a large meta-analysis (126 articles, > 72,000 participants) found marital quality associated with physical health, lower mortality risk and lower cardiovascular reactivity during conflict [37] — a well-being pathway exists in principle, not evidence premarital education produces the marital quality that pathway depends on. None of this literature has measured informed choice, coercion recognition, or safe help-seeking. Two frequently cited works in this literature — Fawcett et al. (2010, *Family Relations*) and Carroll & Doherty (2003) — are not indexed in PubMed and are therefore not cited under this review's PubMed-only citation scope.

E.5 Screening, first-line response, referral, and technology-facilitated routes

The clinical screening literature offers the clearest precedent for the distinction T-PHE's failure criterion is built on. A Cochrane review of 13 RCTs found screening increased identification (OR 2.95) but no evidence of an effect on referral (OR 2.24, 2 studies only) or on re-exposure at 3–18 months [38]. The 2025 USPSTF evidence report reviewed 35 studies ($n = 18,358$): 3 RCTs ($n = 3,759$) comparing screening with no screening found no significant reduction in IPV or other outcomes over 3–18 months, instrument sensitivity ranged 26–87% (specificity 80–97%), and of 13 intervention RCTs only perinatal home-visiting and multi-risk behavioural counselling showed benefit [39]; the companion 2025 recommendation statement retained a moderate-net-benefit judgement for reproductive-age women [40]. Provider training may improve knowledge, attitudes and self-perceived readiness, with weak and inconsistent evidence for actual response behaviour or survivor outcomes [41]. Where first-line response protocols have been trialled directly (WHO LIVES/ARCHES in Nigeria), results are more encouraging but bounded: reduced relative risk of IPV exposure and reproductive coercion among women not already experiencing violence, but no effect on contraceptive use [42], with WHO's own guidance conditioning any inquiry about violence on a functioning referral system already in place [43, 44]. Premarital education itself may function as a gateway rather than a substitute for later help-seeking — one US study found it associated with an increased likelihood of later couples counselling, with ‘not knowing where to go’ reported alongside cost as a principal barrier among low-income couples who did not seek help [106, 107]. Delivery agents themselves may complicate a referral-first safeguard: in a survey of Christian clergy and congregation members, clergy reported being comfortable making referrals for professional counselling, whereas most congregation members preferred counselling with their own pastor and respondents wanted more training on domestic violence — a live tension, not settled support, for a policy that assumes faith-based delivery agents will refer rather than counsel [118].

Evidence on Layer 2 (a proposed extension)'s moderated peer community is genuinely two-sided. On support: a review of youth mental-health peer forums found only 2 of 3 RCTs significant, the other four null [139]; realist syntheses across mental-health and cancer settings agree outcomes depend on moderation, trust and anonymity/privacy control, noting “not all online support groups are helpful, and in some cases they may increase distress” [140, 141, 146]. A scoping review of moderator practice found only 5 of 397 screened articles eligible, only one using a qualified health professional as moderator [142]; qualitative work describes the role as under-supported and under-specified [143], while other qualitative work documents moderators' facilitative strategies and the need for better training and supervision [144]. Weakly moderated platforms specifically amplified harmful content relative to strongly moderated ones [145]. A Chinese IPV-specific peer-support study reports mostly supportive, rarely blaming, content [147], but existing online-IPV tools focus on leaving a relationship with little post-departure attention [148], and digital-IPV-tool safety research remains “virtually non-existent” [137]. On harm: technology-facilitated abuse maps onto every domain of the Power and Control Wheel [149], and more broadly spans digital dating abuse, cyberstalking, technology-facilitated sexual assault, image-based sexual abuse, and online sexual harassment [150]; immigrant/refugee women face additional, unique barriers to escaping and reporting it [151]; it includes systematically characterised cyberstalking [152]; and record-keeping features in coparenting apps have been shown to be sites of both coercive control and survivor resistance [153]. Anonymity increases self-disclosure, while other features of online communication such as asynchronicity, multiple audiences, and audience feedback favor impression management over genuine self-presentation [154]; Muslim youth's social media use carries both cultural/religious connection and discrimination [156]; young people's reported benefits of online help-seeking come with literacy and trust barriers [155]. This symmetry is why Layer 2 needs a named detection mechanism, not only a withdrawal rule.

E.6 Couple-based work, coercive control, and assessor independence

The evidence base most directly relevant to T-PHE's stop rule (R4) is small and explicitly cautious. The only systematic review of couples therapy as an IPV treatment, after screening 1,733 citations, found only 6 studies met inclusion; pooled data suggested conjoint treatment "viable in select situations," with professionals cautious about violent-retaliation risk [45]. Clinical literature converges on screening, safety modification, ongoing reassessment, and distinguishing situational couple violence from coercive controlling violence as prerequisites for any conjoint work [46, 47]. Survivor-voiced evidence complicates any default toward joint response — only 9 of 17 women in one qualitative sample rated couples counselling a good idea [48] — and measurement is contested: coercive control is often reported as reciprocal [49], standard instruments do not measure perpetration and victimisation symmetrically [50], and even validated disclosure instruments substantially undercount coercive control through item-interpretation confusion and fear [51].

No study tests T-PHE's no-stake assessor (a proposed extension); the closest matched evidence is regulated independence in other high-stakes consent settings, offered here as analogy, not domain evidence. Transplant medicine mandates an Independent Living Donor Advocate separated from the recipient's care team [157, 158], extended by proposal to two temporally separated consent processes for domino donors [159]; facial plastic surgery literature names the same conflict-of-interest problem without a mandated remedy [160], and psychotherapy literature names the same dual-relationship problem, noting only meager research and little guidance on nonsexual dual relationships [161], with common occurrence in small or rural settings noted elsewhere [162]. Structured IPV risk-assessment tools show independence helps but does not guarantee accuracy: a meta-analytic review found ODARA best-performing among five compared (weighted AUC=.666), yet only 9 of 20 (45%) predictive-validity estimates used the tool exactly as intended, with a separate study validating an authorised German translation of the ODARA instrument [163, 167]; the Danger Assessment and its 5-item version retain real predictive validity for severe/lethal re-assault (AUC .68–.78) [165]; the Taiwan-adapted version shows similarly strong discriminant and predictive validity, particularly for high dangerousness, though no specific AUC value is reported in the abstract [164]; and a version updated to account for multiple/near-loss-of-consciousness strangulation predicts near-fatal re-assault significantly better than the original Danger Assessment, though no AUC value is reported in the abstract [166]; but multiagency structured-judgment review found it often unclear which recorded factors drove a risk categorisation [168]; a police-administered SARA field study found intervention intensity could reduce recidivism in high-risk cases while increasing it in low-risk cases [169]; structured-judgment ratings, not only actuarial scores, differed by ethnicity between Indigenous and non-Indigenous participants in one study [170]. Check-in and brief-contact models show the same mixed pattern the failure criterion is designed to catch: quality-improvement warm-handoff work doubled successful staff-to-community-health-worker referrals [171], yet an emergency-department screening programme found only 24.5% of screened patients recalled receiving a resource guide despite 58.5% being screened [172]. An SMS brief-contact trial ($n = 804$) found a 22% relative reduction in self-harm repetition at 12 and 24 months but no difference in time to first repeat event [173], and a caring-contacts trial found no significant difference in loneliness at 6 months by treatment arm, concluding an added phone call increased complexity without improving effectiveness [175] (protocol for the related comparative-effectiveness trial: [174]). Implementation-fidelity work in an adjacent prevention programme shows that measured delivery fidelity and dose do not by themselves guarantee outcome change [176]. This literature makes assessor independence necessary but not sufficient, and supports treating the day itself as presence of the space rather than as the outcome — consistent with the failure criterion, not a validation of it.

E.7 Decision quality, autonomy measurement, and digital self-assessment

Decision quality — shared decision-making and informed, values-congruent choice — is this paper's proposed nearest measurement family for T-PHE's decision-control construct (not a validated instrument for it). The "three-talk" model structures shared decision-making into team, option and decision talk [15]; a Cochrane review of 209 decision-aid studies ($> 107,000$ participants) found consistent improvement in knowledge, risk-perception accuracy and values-congruent choice, and reduced decisional conflict [14]. The MRC complex-intervention framework [52], its ADAPT context-adaptation guidance [53], and its process-evaluation guidance [54] supply the methodological scaffolding for describing T-PHE as a programme theory awaiting adaptation and process evaluation, not efficacy testing. The reproductive-autonomy and coercion instruments reviewed above [16, 17, 18] remain the closest content-matched but Western-derived, Thai-unvalidated, family.

Layer 0 (a proposed extension)'s anonymous scored self-check has real precedent, though the precedent does not show benefit. The most rigorously tested example, I-DECIDE, is a masked two-arm RCT ($n = 422$, 79–80% 12-month retention) built from an explicit theoretical model and published protocol [119, 121], with the methodological and ethical challenges of running the web-based trial documented separately [122]; at 6 and 12 months it found no improvement in self-efficacy, with scores in fact slightly lower in the intervention arm than control (mean difference 1.3, 95% CI 0.3–2.3 at 6 months; 1.6, 95% CI 0.5–2.7 at 12 months), and no between-group difference in depressive symptoms [120]. The myPlan app family, tested across protocol, longitudinal and sub-population studies [123, 126, 127, 128], reported a significant reduction in reproductive coercion at 12 months ($p = .019$), but its own reported p -value for suicide-risk improvement ($p = .46$) does not support that outcome as significant despite wording in the source abstract implying otherwise [124]; a bystander sub-study and teen adaptation extend the programme without changing this picture [125, 126]. A double-blind Canadian trial of a tailored online safety and health tool (planned $n = 450$ in the protocol [129], $n = 462$ enrolled in the completed trial [130]) is the closest design analogue to a full self-check-plus-routing model, and a user-centred design report catalogues the wider suite of screening/safety apps this field has produced [131]. The only Thailand-based example, a Thai adaptation of iCanPlan, has so far been evaluated by experts only (usability 4.93/5, cultural appropriateness 5.00/5), not by users [132]; a military-partner survey and two EHR/computer-assisted screening trials extend the pattern without adding a premarital, scored, self-directed instrument [133, 134, 135]. A Kenya-adapted risk instrument shows a scored tool can be validly tailored to a non-Western setting (AUC 0.78) [136], but a systematic review of ICT-based IPV interventions notes safety, equity and unintended-consequence research is "virtually non-existent" [137], and a review of web-based/mHealth IPV interventions finds computer-based/self-administered screening is the most common, not the best-proven, approach [138]. This literature positions Layer 0 as feasible and acceptable, not validated: no record here is evidence a premarital scored self-check changes recognition, help-route knowledge or enacted choice.

E.8 Multicultural clinical safety, language access, and Islamic medical ethics

This is the evidence most directly relevant to T-PHE's cultural-intelligibility safeguard, and it is consistently cautionary. Cultural-competence training for health professionals repeatedly shows provider-level knowledge or attitude gains without a corresponding patient-outcome gain: a Cochrane review of 5 RCTs (337 professionals, 8,400 patients) rated the evidence low-quality, with some improvement in involvement/understanding but no reliable clinical-outcome effect, and adverse outcomes not assessed in any trial [55]; a narrower review (7,879 records to 5 outcome-reporting studies) found no significant patient-outcome improvement in any [56]; and Indigenous cultural-safety training across four high-income countries found outcomes overwhelmingly provider-level, with weak community-involvement and clinical-outcome evidence [57]. Cultural humility [58] and cultural safety — a shift "from provider control to patient agency" [59] — are the constructs this paper adopts for the cultural-intelligibility safeguard, given the pattern above in which content-delivery training alone has not been shown to close this outcome gap. Language-access evidence sharpens the case for interpretation as distinct from cultural content: language discordance is associated with worse outcomes in over half of studies reviewed (84,750

participants) [60] and with higher unplanned readmission/ED revisit [61]; professional interpretation improves patient satisfaction and communication quality relative to ad hoc family/friend interpretation [62], and produces fewer linguistic errors than ad hoc interpretation [63]; and a Canadian cohort ($n = 124,583$) found language-concordant care associated with 36% lower major cardiovascular events among allophone speakers [64]. Thailand-specific evidence situates this in T-PHE's delivery context: interpretation/cultural-mediation for migrant health in Thailand faces budget, training and system-status constraints [65]; migrant-health coverage in Thailand is under-studied in reproductive/maternal domains relative to general health [66]; a Thai maternity-care ethnographic study named "the system is in control," with insensitivity to cultural practice a subtheme [67]; Thai Muslim NCD patients in the rural south report medication-adherence barriers (including Ramadan-related non-adherence) tied to social/cultural context [68]; and Muslim mothers describe faith, patience and family/professional support as their coping structure after their children's cardiac surgery [69]. Migrant/Muslim-women SRH-access reviews consistently name information, language and cultural-difference barriers [70, 20], while quasi-experimental tailored delivery in southern Thailand nearly doubled cervical-screening uptake [71] — a single behavioural endpoint, not evidence that tailoring transfers to T-PHE's decision-control outcome.

Tier-one Islamic bioethics literature gives T-PHE's ethical frame real companions in the medical literature, without becoming evidence the frame, or T-PHE, works. Islamic ethico-legal reasoning has been mapped onto the four principles of biomedical ethics [219] and surveyed as a field spanning virtue ethics to jurisprudential rulings [222, 224]; empirical work with Muslim clinicians describes real moral distress reconciling Islamic and secular institutional ethics and calls for religious-competence training and peer support [221], while cross-gender-interaction norms are addressed separately [220], and end-of-life analyses treat consent as a shared, family-involved process that does not displace individual assent [223]. Two Journal of IMA records, admitted under a labelled professional-association carve-out rather than as leading-journal evidence, frame informed consent as properly incorporating religious/cultural values [235] and frame Qur'an-guided physician character as trustworthy, accountable professional conduct [236]. Faith-integrated delivery can carry a measurable clinical effect: a cluster-randomised mosque-based diabetes-prevention trial reduced 12-month type 2 diabetes incidence from 17.1% to 9.8% (adjusted hazard ratio 0.75, 95% CI 0.60–0.95, $P = .02$) [225], and a mosque-based, lay-led Islamic Trauma Healing programme for refugee PTSD showed effect sizes of $d = -0.67$ (PTSD severity), $d = -0.66$ (depression) and $d = 0.71$ (well-being), maintained at 12-week follow-up [226] — though two published errata (*JAMA Netw Open* 2024;7(9):e2440198 and 2024;7(10):e2446257) mean the corrected article, not the original abstract alone, should be checked before any number from it is repeated. A religiosity-anxiety meta-analysis found a consistent protective association (mean $r = -0.22$ across 10 studies) [232] (further tier-one companions on vaccination decision-making, critical-illness family needs and faith-based infectious-disease framing: [227, 228, 237]). This literature also complicates any simply-protective reading: a systematic review of gender-based violence among migrant Muslim women found the husband's control over the woman's life a risk factor for violence at the microsystem level, even where spirituality was protective at the individual level [229], and a systematic review of self-immolation found marital and familial conflict a principal driver in some Muslim-majority settings [233] (further tier-one companions on sexual/reproductive health, assisted reproduction and religion-and-health: [230, 231, 234]). No indexed FIMA or IMANA premarital guideline and no Thai Muslim premarital trial exist in this record. This is what the abstract's ethical terms are declared alongside, in their own field's own words, with a plain gloss attached — the frame's companion literature, never evidence that the frame prevents harm.

E.9 Thailand and the southern border provinces

Direct Thai evidence remains sparse and geographically uneven. A four-region national survey ($n = 2,462$ married/cohabiting women) found roughly 15% lifetime psychological/physical/sexual IPV and controlling behaviours ranging 4.6–29.3% [72]; clinic-based studies found 21% past-year IPV across two Bangkok gynaecology clinics (IPV associated with STI, aOR 2.65) [73] and 13.1% ever-abused among 475 pregnant Thai women [74]. Provider-side evidence — from *northeast* Thailand, not the southern border — found providers valued IPV screening but cited cultural-norm, training and service-condition barriers [75], a caution against generalizing provider attitudes across Thai regions. Southern-border-specific evidence is more extensive: among 700 Muslim adolescents on the southern border, 9% were sexually experienced, with condom non-use, STI and repeat pregnancy concentrated in that subgroup [76]; among 360 Muslim army conscripts in the three deep southern provinces, 78.7% showed poor HIV/STI-transmission knowledge, examined further through an Islamic-values framing of sexual knowledge in a related conscript sample [77, 78]. Mental-health evidence from the conflict-affected southernmost provinces is consistent in direction across cohorts: migration and economic stress were associated with more psychiatric symptoms and religiosity with fewer among 2,053 Muslim adults [79]; among youth 18–24, parental absence and travel-safety worry were associated with more psychiatric symptoms, with religious practice protective for men [80]; among 387 adolescents, 7.2% screened at risk of depression, with family dysfunction and PTSD strongly associated with depression risk [81]; and young adults wishing to migrate for work reported more psychiatric symptoms and lower happiness, with chronic violence linked to education, employment and settlement plans [82]. The same religious-interpretation and trust structure recurs in vaccine-acceptance evidence: per interviewed health workers, perceived religious prohibition, spousal disagreement, fear of side effects and religious-leader views were the leading barriers to MMR-vaccine acceptance among Muslim parents [83], and childhood-immunization beliefs were shaped by self-interpretation, misinformation and halal concerns, tempered by reliable information and community health-network leaders [84]. No study identified in either round evaluates a premarital-education programme, of any design, with a Thai Muslim or Thai intercultural population.

E.10 Culture as a health determinant, humility, and self-regulation

Framed at the widest level, culture-and-health scholarship supplies theoretical warrant, not efficacy evidence, for treating culture as a determinant T-PHE's safeguards must hold rather than resolve. The Lancet Commission on Culture and Health argues culture shapes the meaning of health, behaviour, trust and communities of care [85]; a companion Lancet series maps mechanisms — separation and hierarchical power [86], and pathways through health systems and communities [87] — by which racism and discrimination affect health, and a 2026 follow-up renews calls for inclusive services, equitable access, rights and dignity for people on the move [88]. Evidence on whether *adapting* an intervention to a specific culture adds benefit beyond a generically delivered one is more equivocal than commonly assumed: a meta-analysis of 67 RCTs of culturally adapted psychological interventions for people of Chinese descent found a medium overall effect but *no significant difference* between culturally modified and culturally specific adaptation types [89] — evidence against assuming any particular adaptation strategy is superior, not against cultural adaptation itself. Two structured-social-support RCTs outside the marriage-transition setting offer an analogy, not evidence, for the mechanism T-PHE's safeguards are built around: a Nepal social-capital-building programme was associated with lower incident major depressive disorder (7.4% vs 12.9%) [90], and a UK family-wellbeing RCT (674 participants, 62% ethnic-minority) improved parental mental wellbeing post-intervention and at 6 months [91]. The pattern is read here as suggestive that structured social support, not content delivery alone, may be the active ingredient — a hypothesis adopted by analogy, not one either trial tested directly.

The constructs the model uses to gloss *faqr* and *taqwa* — humility, compassion and self-regulation — are real, measured psychological variables, but the evidence sits mostly in general and clinical populations, not premarital or Muslim ones, and the sequence to safety is not self-executing. Intellectual humility predicts more constructive responses to conflict [198], greater empathic accuracy toward outgroup members [199], and greater adherence to expert recommendations even controlling for political affiliation [200]; cultural humility has a

developed theoretical basis [201, 202] and institutional training improves attitudes and knowledge, with the authors themselves cautioning that a single training session is insufficient [203]. Self-compassion has a large protective association with psychopathology ($r = -0.54$, 95% CI -0.57 to -0.51) [204], and compassion-focused therapy improves self-compassion and reduces symptoms in trial evidence [205], but its relational benefit is conditional: among men, self-compassion predicted constructive repair only when conscientiousness was high, and the association reversed at low conscientiousness [206]; two further studies link self-compassion to relationship satisfaction and conflict resolution in general and Black heterosexual-couple samples [207, 208]. Empathy shows the same double edge: working memory was associated with less physical abuse perpetration, an effect mediated by affective empathy [209], but in one clinical sample of men, emotional empathy was positively associated with perpetrated psychological IPV [210] — restraint in the use of power, not empathy alone, is the load-bearing step. Self-regulation deficits (inattention, impulsivity) are cohort-study risk factors for adult IPV perpetration, and existing treatment programmes are reported to have minimal impact to date [211]; a systematic review traces the broader psychosocial pathway from childhood intra-familial victimisation to adult relationship violence [212]. Religiosity and spirituality were protective for general interpersonal aggression but *not* specifically for domestic violence in a 67-study systematic review and meta-analysis [215], complicating any simple “faith protects” reading (further tier-one companions extending without resolving this picture: [216, 217]). Trust and dyadic-maintenance scales [213, 214], a humility-forgiveness mediation study [197], and the only premarital analogues in this literature — an 8-couple feasibility pilot of a culturally adapted couple-learning programme [100] and a premarital HIV-counselling cluster RCT [218] — round out the picture. Humility and compassion are evidence-bearing constructs by analogy from therapy and general populations; they position R1’s ethical language, they do not supply evidence that R1 works in this setting.

E.11 Gap statement

T-PHE is offered as a programme theory — a structured account of the mechanism by which a governance layer of five rules (R1–R5), including a stop–assess–refer–record rule (R4), is hypothesized to preserve or restore each participant’s decision control at the marriage transition — and not as a validated or effective intervention. No component of T-PHE has been tested, in any population, against the outcomes its own failure criterion names as necessary (informed choice, coercion recognition, reproductive and mental-health literacy, safe help-seeking, accountable referral). This review adds two findings that sharpen, without resolving, the case for testing: first, that a premarital contact point can plausibly be built and delivered at population scale (drawing on preconception medicine and premarital genetic screening as the nearest analogue, not a tested precedent for T-PHE itself), while knowledge gained at that contact point does not reliably convert into the behavioural or decision outcome desired, in either the screening or the relationship-education literatures; second, that cultural-competence and cultural-tailoring interventions elsewhere in health care repeatedly raise provider- or participant-level knowledge and attitudes without a demonstrated patient-outcome gain, and that no adaptation strategy has been shown superior to another in the one large trial that tested this directly. No existing instrument for any of T-PHE’s target outcomes has been validated in a Thai or Thai Muslim population, and the decision quality/shared-decision-making measurement family this paper proposes as the nearest analogue for “decision control” is itself an unvalidated adaptation, not a borrowed instrument. T-PHE should be read, and should be tested, as a falsifiable synthesis awaiting empirical evaluation — never cited, by its authors or by others, as evidence that premarital education of this design improves, prevents, or reduces any outcome.

F Illustrative scenario modelling (not evidence)

F.1 Delivery conditions from a simulation output

A person-level Monte Carlo simulation was run under declared assumptions — two independent risk draws per couple, classroom-level facts logged once per classroom, a single assessor draw per case, and pre-arranged versus day-of logistics as one switch — at 100 classrooms (eight couples each, five seeds) and again at 1,000 classrooms (three seeds). This is a result of the assumptions coded into the model, not empirical data about any delivered programme: no participant was observed, and nothing here is an effectiveness estimate. It is reported because it shows which operator-controllable conditions the model’s own assumptions make consequential, checked by an independent adversarial re-run across additional seeds.

That re-run confirmed the rankings from the original seeds while showing the first point estimates were seed-specific and should be reported as ranges. Across seeds 1–8 and both scales, in the simulation, under its assumptions, pre-arranging an assessor with no stake in the wedding, a private room, and a professional interpreter *before* the day was the single largest lever: true-risk cases handled safely rose ≈ 2.1 – $2.5\times$, cases left stuck with no assessor fell 2.5 – $4.9\times$, and private slots overheard by family fell 6 – $10\times$. These rankings held across seeds and at ten-fold scale; the magnitudes did not, hence the ranges.

Two further conditions followed. In the simulation, under its assumptions, once logistics were pre-arranged, untrained facilitators became the dominant residual cause of unsafe continuation. In the simulation, under its assumptions, a narrow, trained trigger list reduced false triggers relative to a broad one. Day length behaved differently: in the simulation, under its assumptions, walk-outs ($\approx 5\%$ of couples, modelled) were unchanged by any safeguarding lever and moved only when the dose — the length of a single day — was redesigned, for example by moving content to a follow-up slot. Table 6 summarises the four conditions the output ranked as decisive for delivery, as distinct from the model elements themselves. None of this is a finding about T-PHE in practice; it states what the declared assumptions in the simulation code imply, offered as input to co-design, not as evidence that any condition improves an outcome.

Table 6: Four delivery conditions the simulation output ranked as decisive (not model elements; output of assumptions, not data). All figures are simulation outputs of declared assumptions, not measured effects; the assessor row’s “no stake” condition is a proposed extension supported only by analogy evidence ([157, 158, 161, 162]; see Results, Table 1). A case the model counts as “stuck” is one where the R4 referral pathway — to a qualified multidisciplinary service such as Thailand’s One-Stop Crisis Centre (OSCC) or an equivalent designated service — could not be completed in the simulation’s own assumptions, not one in which OSCC access itself was tested.

Condition	What the output attributes to it, in the simulation
Assessor, room, and interpreter pre-arranged before the day	Largest lever modelled: safely handled cases $\times 2.1$ – 2.5 ; stuck cases $\div 2.5$ – 4.9 ; overheard private slots $\div 6$ – 10 .
Facilitator training gate before delivery	Once logistics are pre-arranged, the residual cause of unsafe continuation in the model.
Narrow, trained trigger list	Fewer false triggers than a broad or untrained list, in the model.
Follow-up slot rather than a longer single day	The only lever in the model that moved walk-outs, which safeguarding levers alone did not change.

G Version history

Version	Date	Principal change
Abstract	2026 (accepted)	Camera-ready abstract, TIMA/FIMA Scientific Convention 2026; document of record in the RE_T-PHE repository.
Note v1.0	2 Sep 2026	Full paper note expanded from the abstract (17 sections, 5 appendices).
Note v1.1	2 Sep 2026	Ethical root corrected to faqr $\rightarrow$ taqwa $\rightarrow$ amanah; shura repositioned as method; 'adl as test.
Paper v0.1	2 Sep 2026	This IMRAD skeleton, two-column preprint format. Private until independent critical appraisal.
Paper v0.2	3 Sep 2026	Purpose-review pass (audit/purpose-review.md): six-safeguards/five-rules mapping stated explicitly; Institutional position and Invitation to co-design subsections written; Novelty and standpoint subsection added; OSCC named in the referral pathway; AI/no-participant-data wording unified; §4.2’s literature narrative moved intact to Appendix E, replaced with a condensed synthesis; remaining placeholder markers resolved or removed.
Paper v0.4	3 Sep 2026	Merged independent adversarial review pass (public-health, clinician, judge; audit/v0.4-editlist.md): affiliations restored exactly as in the submitted abstract; filled the Reporting-guideline subsection; the failure-criterion register corrected to “pre-specified” throughout, with no registry-deposit term used anywhere; R4 assessor-unavailable default, an emergency/imminent-danger pathway, a named safeguarding lead, assessor dual-relationship independence, a data-handling protocol, and a minors/mandatory-reporting boundary added to the safeguarding specification; the scenario-simulation section moved out of Discussion into Appendix F (“Illustrative scenario modelling, not evidence”), disclosed in Methods, and its vocabulary de-jargoned; WHO RESPECT citation reconciled to the 2025 second edition; Phase 1 co-design design detail and governance/ownership/partner-risk sentences added.

Table 5: Evidence map for propositions P1–P7 (reference numbers as in the bibliography; each entry carries its PMID).

	Relevant / complicating records	Evidence level	What remains untested
P1	<i>Relevant:</i> burden/instrument base [1, 2, 5, 16, 18, 92]; norms (not knowledge) drove 70–95% of the SASA! effect [26, 25]; decision-aid/shared-decision-making analogue [14, 15]. <i>Complicating:</i> RCT-only aggression effect not convincing [6, 33, 7, 8]; uneven knowledge/literacy-to-outcome transfer [34, 35]; Muslim identity does not uniformly predict lower autonomy [93].	Mixed: burden data and single-time-point instrument-validity studies are strong; the causal claim that a governance layer <i>adds</i> decision control beyond content, and that decision-quality measures capture it, is untested for any premarital-education format.	Whether a governance/safeguard layer added to premarital content produces a measurable decision-control effect distinct from content alone, using any instrument (existing or adapted), has not been tested.
P2	<i>Relevant (adjacent precedent only — no cited record tests a private channel specifically inside premarital/marriage education):</i> [94, 95, 96, 97, 98, 99]. <i>Complicating:</i> even validated instruments substantially undercount coercive control [51].	Mixed: instrument-validation and single trials; retention/attrition data are a direct caution.	Whether a private channel embedded specifically in premarital education (not a clinical encounter) reliably surfaces concealed coercion has not been tested.
P3	<i>Relevant:</i> [100, 101, 27]; tailored edutainment nearly doubled cervical-screening uptake, a single behavioural endpoint [71]. <i>Complicating:</i> [102, 103, 19]; cultural-competence training repeatedly improves provider knowledge/attitude without patient-outcome gain [56, 57, 55]; no efficacy difference between adaptation types in one large meta-analysis [89].	Mixed to low: mostly single small trials, pilot studies, and systematic reviews of provider-level rather than patient-level outcomes; no controlled comparison of tailored vs untailored delivery on a safety outcome.	Whether cultural tailoring can be operationalized so that it improves engagement without ever being permitted to override a consent/safety threshold has not been tested in any premarital-education trial.
P4/ STOP	<i>Relevant/analogous (not effectiveness support):</i> [45, 46, 47, 48, 9, 10, 43]. <i>Complicating:</i> [49, 50].	Bounded: the strongest single review here is a meta-analysis of only 6 small studies.	No study has tested T-PHE’s specific stop rule (R4) against a graduated-response alternative; the assessment mechanism that would trigger it is not specified or validated in T-PHE’s own materials.
P5	<i>Relevant:</i> 2025 USPSTF evidence report and recommendation statement [38, 39, 40]; referral pathway [42, 43, 44, 104, 105, 106, 107]; language-concordant referral [60, 62, 63, 61, 64]. <i>Complicating:</i> provider training improves knowledge/readiness, not reliably behaviour [108, 109, 41].	Strong for the general principle; weak/absent for T-PHE’s own referral pathway, and untested for whether it meets language/cultural-accessibility standards in a Thai setting.	Whether T-PHE’s proposed referral pathway meets timeliness/accessibility/follow-through <i>and</i> language/cultural-accessibility thresholds in a Thai setting has not been tested.
P6	<i>Relevant:</i> [110, 28, 111, 112]; a faith-leader trial found no effect on its primary outcome (secondary IPV effects only) [29]; adjacent-field confirmation: screening uptake “suboptimal” despite an established programme [12]; knowledge gains consistent, uptake inconsistent [13]; RHL gains uneven across dimensions [34]; knowledge/attitudes rose, no clinical outcomes reported [35]; provider training raises knowledge/attitude without patient-outcome gain [55, 56, 57].	Strong: multiple RCTs and systematic reviews across two literatures now directly demonstrate this knowledge/behaviour and process/outcome dissociation.	Whether this dissociation pattern specifically occurs in T-PHE’s own programme has not been tested — it is now well-grounded across two adjacent literatures, not yet observed in T-PHE.
P7	<i>Relevant:</i> [113, 114, 31, 115, 116]; methodological scaffolding for future revision cycles: MRC complex-intervention framework [52], ADAPT guidance [53], MRC process-evaluation guidance [54].	Weak to moderate: mostly programme-description and narrative-review evidence, plus general-purpose methodological guidance, not outcome trials of repair mechanisms themselves.	Whether T-PHE has, or will build, an actual mechanism (audit cycle, feedback loop, revision trigger) for reparability has not been specified or tested.